

Answers CMS - Obstetrics and Gynecology - Form 1

1. **A** - WBC high + right lower quadrant tenderness + negative urinary nitrates and no costovertebral angle tenderness to rule out pyelonephritis = appendicitis.
2. **A** - Turner syndrome typical characteristics, FSH will be high. No (streak) ovaries mean the body will try to stimulate estrogen production with FSH, but won't be able to.
3. **C** - Prolonged cystitis symptoms often with negative urinary culture = interstitial cystitis.
4. **D** - Sertoli-Leydig cell tumor. **Q:** Why not PCOS? **A:** There is no solid mass in PCOS but multiple cysts. Plus there are no genital changes in PCOS, but due to high testosterone in a Sertoli-Leydig tumor, this patient has genital changes
5. **E** - An incisional seroma is a collection of lymphatic fluid in an area of former dead space that may require drainage tubes. This patient's incision is dry.
6. **D** - Submucosal myomas that protrude into the uterine cavity are most frequently related to significant heavy menstrual bleeding. One theory is that it inhibits the usual ability of the uterus to contract during menses. Submucosal or intramural fibroids result in difficulty conceiving a pregnancy and an increased risk of miscarriage (UTD).
7. **D** - Thyroid Storm in Pregnancy Tx: 1.) Propranolol, 2.) Propylthiouracil, 3.) Steroids (dexamethasone). We do not give potassium iodide in pregnancy as it may affect the fetal thyroid gland (**Note:** there have been studies in Japan cited on UTD suggesting that Iodide during pregnancy may result in worse thyroid control but lower morbidity than using PTU, i.e. this is not a great question). We never do radioactive iodine thyroid ablation in pregnancy either.
8. **D** - She has a history of preterm labor, which predisposes her to it again. In pregnancy history tends to repeat itself (previous preterm labor is one of about 25 risk factors for preterm labor). Also while pregnancy outcomes with bicornate uterus have been reported to be close to those of the general population, some women do develop complications, such as pregnancy loss, preterm labor, or malpresentations (UTD).
9. **E** - Hyperemesis gravidarum that's pretty serious since she can't eat or drink and is dehydrated.
10. **A** - No satisfying explanation yet as to her underlying condition (atelectasis is a symptom, not really a condition).
11. **B** - Classically estrogen is believed to reduce milk production. However, two 2015 updated systematic reviews, with minimal overlap in included studies, both concluded that the data on the impact of estrogen-progestin contraceptives on breastfeeding duration and success was inconsistent and limited (i.e. this is not a great question) (UTD).
12. **E** - Lichen sclerosis is premalignant so a punch biopsy is indicated then application of steroid cream for symptomatic relief (of LS, not if it turns out to be malignant)
13. **C** - Atrophic vaginitis is one possible cause of postmenopausal bleeding in postmenopausal women.
14. **E** - Rhogam to prevent alloimmunization following the prenatal bleeding.
15. **E** - This is a molar pregnancy due to the high hCG and the scattered hyperechoic material (often described as "snowstorm" appearance). Treatment of choice is with a suction curettage.

16. **D** - After puberty, pubic hair and breast changes happened already and it tells that her periods are about to happen soon (Thelarche, Adrenarche, Growth Spurt, Menarche). **Q**: Why not E, Adrenarche, considering her weight is 90th percentile? **A**: Precocious puberty is in girls <8 years.

17. **E** - Uteroplacental insufficiency is one of the many risk factors for IUGR.

18. **A** - Breast engorgement causes fever and is non-pathological. **Q**: Why not mastitis? **A**: Her WBC count is normal, and mastitis is usually unilateral, not bilateral as in this patient. **Q**: Why not cystitis? **A**: 0-5 WBCs on the UA is generally considered within normal limits, and she has no urgency/frequency. Mild pain with urination and presence of RBCs on the UA is likely due to recent removal of a Foley (since she had a C-section).

19. **C** – Lidocaine overdose, or lidocaine absorbed into the circulation, can cause toxicity, which is a serious and potentially fatal complication. Signs of lidocaine toxicity include numb tongue or lips, tinnitus, bad taste in the mouth, nausea, and sedation.

20. **D** - 5 α -reductase is most known for converting testosterone, the male sex hormone, into the more potent dihydrotestosterone. So more potent dihydrotestosterone = more hairs.

More:

General Rules: Testosterone comes from the ovaries, DHEA comes from the adrenal glands, 17-hydroxyprogesterone comes from the adrenal glands (mid-step product that appears in the urine in CAH).

Work Up: For any patient coming with hirsutism or virilization symptoms, Next Best Step:

- Laboratory Testing:
 - Testosterone
 - If abnormal → lesion is ovarian, do ultrasound →
 - If lesion is bilateral, likely PCOS, do LH/FSH level to confirm
 - If lesion is unilateral, likely granulosa-theca cell tumor (secretes estrogen)
 - DHEA
 - If DHEA markedly elevated, likely adrenal tumor → do CT scan abdomen to check for "unilateral" lesion and confirm with adrenal vein sampling
 - If DHEA mildly elevated, likely congenital adrenal hyperplasia (CAH) → do CT of abdomen to check for "bilateral" lesion and confirm with elevated 17-hydroxyprogesterone level
 - 3-17-hydroxyprogesterone
 - If elevated, confirm congenital adrenal hyperplasia (CAH).
- If you perform all initial laboratory test and all levels are within normal (as in our case in this question), then you are dealing with idiopathic type
 - Rationale: the existence of 5 α -reductase activity (responsible of the conversion of testosterone into the very potent form of dihydrotestosterone – DHT) produces certain features in both genders secondary to high levels of DHT
 - Male
 - Pre-natal (results in external male genitalia)
 - Post-natal (results in scalp hair loss, increased hair in the axillary + pubic and extremities + prostate enlargement)

- Female
 - Results in growth of hair on arms and legs beside axillary and pubic areas.

Conclusion: The excessive hair growth in this girl in the absence of lab abnormalities, is suggestive of increased 5 α -reductase activity (which is the idiopathic)

21. **A** - The presence of dark bleeding + pain (significant contractions) is suggestive of abruptio placentae complicating preterm labor (regular contractions every 1-2 minutes). If E (idiopathic preterm labor) was changed into “preterm labor secondary to abruptio placentae”, then it should be a great alternative answer as well.

22. **C** - She has a tender fundus and a fever.

23. **E** - Note that you cannot give TMP-SMX during pregnancy as it is a teratogen.

Continuous	Postcoital (within 2 hours of coitus)
Trimethoprim/sulfamethoxazole (TMP/SMX) (40 mg/200 mg daily or thrice weekly) ⁴⁵	TMP/SMX (40 mg/200 mg to 80 mg/400 mg) ⁴⁶
Trimethoprim (100 mg daily) ^{45, 47}	
Ciprofloxacin (125 mg daily) ⁴⁸	Ciprofloxacin (125 mg) ⁴⁸
Cephalexin (125 mg to 250 mg daily) ⁴⁹	Cephalexin (250 mg) ⁵⁰
Cefaclor (250 mg daily) ⁵¹	
Nitrofurantoin (50 mg to 100 mg) ⁵²	Nitrofurantoin (50 mg–100 mg daily) ⁴⁶
Norfloxacin (200 mg daily) ⁵³	Norfloxacin (200 mg) ⁵⁶
Fosfomycin (3 g every 10 days) ⁵⁵	
	Ofloxacin (100 mg) ⁵⁶

24. **C** - This sounds like a retained urogenital septum rather than an imperforate hymen since you can visualize into the vagina, but just not the cervix. In either case, it has led to an accumulation of menses (hematocolpos) since she underwent menarche.

25. **E** - Primary dysmenorrhea refers to the presence of recurrent, crampy, lower abdominal pain that occurs during menses in the absence of demonstrable disease that could account for these symptoms. Secondary dysmenorrhea has the same clinical features, but occurs in women with a disorder that could account for their symptoms, such as endometriosis, adenomyosis, or uterine fibroids (UTD).

26. **B** - Müllerian abnormalities affect both the uterine-tube system and the renal system. She doesn't have Müllerian agenesis, otherwise she'd have no uterus or upper vagina.

27. **E** - This is a classic presentation of *S. aureus*-mediated Toxic Shock Syndrome probably due to a retained tampon.

28. **A** - This is Mittelschmerz, a benign condition where ovulation causes recurrent midcycle pain in females with regular ovulatory cycles. This pain is caused by normal follicular enlargement just prior to ovulation or to normal follicular bleeding at ovulation. The pain is typically mild and unilateral; it occurs midway between menstrual periods (i.e. days 13-14) and lasts for a few hours to a couple of days (UTD).

29. **L**

30. **B** - This is highly suggestive of thalassemia. The biggest clues are that she is Asian and her low hematocrit despite iron supplementation.

31. **C** - "Family history" is really only significant for 1st degree relatives. Her great aunt is her grandparent's sister, thus it has almost no contribution to her risk.

32. **C** - The bleeding started after intercourse, which provides a big clue. One big distractor is the option placenta previa, which can easily bleed upon intercourse or pelvic examination. Beware! The stem mentioned fundal placenta was documented on 20 weeks' gestation ultrasonography (this rules out the possibility of placenta previa - once fundal placenta confirmed earlier this option is out). **Q**: Why it is not cervical cancer? **A**: Usually cervical cancer in pregnancy would be detected earlier on Pap smear, and in the stem of the question it was mentioned that the patient had uncomplicated pregnancy so far. Only after this recent physical incident did she start to have spotting, which is a very specific finding of cervical lesion. If it was cervical cancer, the bleeding should be spontaneous, and would have been detected on Pap smear. Other symptoms may also be observed: symptomatic stage IB disease presents with abnormal vaginal bleeding or discharge; patients with more advanced disease also presented with pelvic pain, sciatica-type leg pain, flank pain, chronic anemia, and shortness of breath (UTD).

33. **B** - This patient likely has PCOS. The enlarged uterus could either be the result of endometrial hyperplasia from unopposed estrogen or fibroids fed by the estrogen.

34. **B** - On the test, treat appropriately, regardless of cost (i.e. her insurance running out in 2 weeks is a non-factor in choosing a therapy).

35. **D** - Almost always, diagnose before you treat.

36. **E** - This is a classic scenario and image for *Neisseria gonorrhoeae*, showing the G- diplococci. *Chlamydia trachomatis* is a more common cause of mucopurulent cervicitis than *Neisseria gonorrhoeae*, but with the image the identity is clear (UTD).

37. **H** - Last menstruation 6 weeks ago with regular menstruation prior, and "thin, clear" vaginal discharge, "friable" cervix, increased urinary frequency, and enlarged uterus are all typical symptoms of pregnancy along with physiological leukorrhea of pregnancy.

38. **A** - This is a classic vignette describing trichomoniasis with strawberry cervix, elevated pH (>4.5), and yellow-gray discharge. Only the fishy odor doesn't fit exactly.

39. **E** - She has diastolic BP >90 and several other features of "severe preeclampsia", now called preeclampsia with severe features (UTD).

Criteria for the diagnosis of preeclampsia

Systolic blood pressure ≥ 140 mmHg or diastolic blood pressure ≥ 90 mmHg on two occasions at least four hours apart after 20 weeks of gestation in a previously normotensive patient
If systolic blood pressure is ≥ 160 mmHg or diastolic blood pressure is ≥ 110 mmHg, confirmation within minutes is sufficient
and
Proteinuria ≥ 0.3 g in a 24-hour urine specimen or protein/creatinine ratio ≥ 0.3 (mg/mg)(30 mg/mmol)
Dipstick $\geq 1+$ if a quantitative measurement is unavailable
OR
New-onset hypertension with the new onset of any of the following (with or without proteinuria):
Platelet count $< 100,000/\text{microL}$
Serum creatinine > 1.1 mg/dL (97.2 micromol/L) or doubling of the creatinine concentration in the absence of other renal disease
Liver transaminases at least twice the upper limit of the normal concentrations for the local laboratory
Pulmonary edema
Cerebral or visual symptoms (eg, new-onset and persistent headaches not responding to usual doses of analgesics; blurred vision, flashing lights or sparks, scotomata)

40. **A** - You either memorized this for STEP1 or you didn't.

41. **E** - The initial presentation of HSV2 can be severe with painful genital ulcers, dysuria, fever, tender local inguinal lymphadenopathy, and headache. In other patients, however, the infection is mild, subclinical, or entirely asymptomatic. Other symptoms and signs in these first episode infections included systemic symptoms, including fever, headache, malaise, and myalgias (67%), local pain and itching (98%), dysuria (63%), tender lymphadenopathy (80%). Symptoms tended to be more severe in women than in men. Clinical recurrences of genital HSV are common (what this question is describing), but are typically less severe than primary or nonprimary infections. The mean duration of lesions is generally shorter with recurrences than in primary infection (10 versus 19 days) and the duration of viral shedding is usually two to five days. Systemic symptoms are infrequent and approximately 25% of recurrent episodes are completely asymptomatic. As many as 50% of patients with symptomatic recurrences have prodromal symptoms before eruption such as local mild tingling or shooting pains in the buttocks, legs, and hips (UTD).

42. **C** - This presentation is typical for placenta previa. Check fetal ultrasound and never perform pelvic/digital/cervical examination until placenta previa has been ruled out.

43. **C** - In gastroschisis, the intestines are not covered by a membrane; in an omphalocele they are covered by peritoneum.

44. **E** - These are variable decelerations, which are caused by cord compression. Helpful tip from our clerkship director: cover up the contractions and decide if the decelerations are variable (pointy) or early/late (smooth, not pointy), then go from there, because they will give you variables with each contraction that line up with the nadir of the contraction and try to trick you into guessing early decelerations (there is a UWorld question on this), but the pointy shape means it's a variable deceleration, regardless of how it lines up with the contractions.

45. **E** - The progesterone withdrawal test was negative, so it probably means she's not making estrogen (a withdrawal bleed means she's making estrogen but not progesterone due to anovulation). It's likely to be a hypothalamic cause, and it could be anorexia, stress or exercise. These factors cause a decrease in GnRH leading to a decrease in estrogen. Low estrogen increases her risk for osteoporosis.

46. **D** - Recurrent vulvovaginal candidiasis is defined as four or more episodes of symptomatic candidal vaginitis in a 12-month period. Attempts should be made to eliminate or reduce risk factors for infection if present (e.g. improve glycemic control, switch to lower estrogen dose oral contraceptive) (UTD).
47. **A** - Type 2 DM is a risk factor for macrosomia, which should be suspected with her fundal height 4 cm exceeding her estimated gestational age. The baby is in the proper position (vertex, occiput anterior).
48. **A** - She has stress incontinence (leakage occurs with increased abdominal pressure, e.g. laughing or sneezing). This is caused by decreases external urethral sphincter tone.
49. **F** - Lactational mastitis is a condition in which a woman's breast becomes painful, swollen, and red; it is most common in the first three months of breastfeeding. Initially engorgement occurs because of poor milk drainage, probably related to nipple trauma with resultant swelling and compression of one or more milk ducts. If symptoms persist beyond 12 to 24 hours, the condition of infective lactational mastitis develops (since breast milk contains bacteria). Infective lactational mastitis typically presents as a firm, red, painful, swollen area of one breast associated with fever $>38.3^{\circ}\text{C}$ in a nursing mother; milk secretion may be diminished. Systemic complaints may include myalgia, chills, malaise, and flu-like symptoms. In the early stages, the presentation can be subtle with few clinical signs; patients with advanced infection may present with a large area of breast swelling with overlying skin erythema. Reactive axillary lymphadenopathy may be associated with axillary pain and swelling (UTD).
50. **K** - Bleeding can occur in early pregnancy due to the following factors: Implantation bleeding can occur anywhere from 6-12 days after possible conception. Every woman will experience implantation bleeding differently - some will lightly spot for a few hours, while others may have some light spotting for a couple of days. Some type of infection in the pelvic cavity or urinary tract may cause bleeding. After intercourse, some women may bleed, because the cervix is very tender and sensitive. In this case a woman should discontinue intercourse until she has been seen by her doctor. This is to prevent any further irritation - having normal sexual intercourse does not cause a miscarriage. Signs of miscarriage include vaginal bleeding (usually heavy, a period or more), cramping pain felt low in the stomach (stronger than menstrual cramps), tissue passing through the vagina.

In next pages, you will find the answers made by someone else but the order of answers is not same. I have left it there in case it helps you better understand the answers. I didn't find either of the explanations helpful

NBME OB/GYN Form 1 - Answers & Explanations

Updated: Jul 27, 2020

1) Cephalopelvic disproportion

- Second stage arrest of labor (no fetal descent after pushing for ≥ 3 hours (nulliparous) or ≥ 2 hours (multiparous))
- The optimal fetal position is occiput anterior; If the fetus is in occiput anterior, the most likely cause of second stage arrest of labor is cephalopelvic disproportion
- Montevideo units (MVU) = mmHg (60) x # of contractions in 10 mins (5) = 300 (MVU ≥ 200 is adequate)

2) Primary dysmenorrhea

- Crampy lower abdominal and back pain during menses with a normal examination
- Tx: NSAIDs if sexually inactive, OCPs if sexually active

3) *Staphylococcus aureus*

- Toxic shock syndrome likely due to prolonged tampon use $\rightarrow$ systemic inflammatory response to toxic shock syndrome toxin-1, an exotoxin that acts as a superantigen

4) Fetal ultrasonography

- This patient has received no prenatal care in the past. Placenta previa needs to be ruled out (painless bleeding that can otherwise be Asx)
- Placenta previa must be ruled out prior to a digital exam

5) Submucosal

- Submucosal fibroids are closer to the endometrial lining, and thus are more prone to bleed

6) Oral contraceptive therapy and a second pelvic examination in 6 weeks

- Simple cysts in premenopausal women usually resolve on their own; hormonal contraceptives inhibit ovulation and thus prevents the formation of new cysts

7) Flagellated protozoa

- Trichomoniasis is characterized by thin yellow-green discharge and a vaginal pH > 4.5
- Tx: Metronidazole for patient and sexual partner

8) Gastroschisis

- Extrusion of abdominal contents, typically to the right of the umbilicus
- Not covered by peritoneum or amnion

9) Breast engorgement

- Engorgement - Bilateral symmetric fullness, tenderness and warmth (this patient's lack of breastfeeding $\rightarrow$ engorgement)
- Mastitis - Unilateral, localized tenderness and erythema (additionally, this patient is not breastfeeding, so the likelihood of her getting mastitis is low)

10) Autosomal dominant

- Achondroplasia has an AD inheritance pattern

11) Normal pregnancy

- Normal to have implantation bleeding in 1st trimester
- Placenta previa usually presents with sudden and profuse vaginal bleeding after 28 weeks of gestation

12) Hormone therapy

- HRT $\rightarrow$ $\uparrow$ risk of endometrial and breast cancer

13) Hematocolpos

- This adolescent girl likely has an imperforate hymen - amenorrhea and cyclic lower abdominal pain with bulging vaginal mass (hematocolpos)

14) Reassurance

- This patient likely has Mittelschmerz, transient mid-cycle ovulatory pain that may mimic appendicitis
- Reassurance is indicated once acute pathology is excluded

15) Preterm labor and delivery

- This patient has a Hx of preterm labor ($\rightarrow$ $\uparrow$ risk of future preterm labor)

16) Ureterolithiasis

- Flank pain radiating to the groin is consistent with lithiasis
- No gross hematuria, but there could be microscopic hematuria indicative of lithiasis
- Fever would be present if this patient had pyelonephritis

17) Osteoporosis

- No withdrawal bleeding = absent/low estrogen
- Estrogen deficiency $\rightarrow$ $\uparrow$ risk of osteoporosis

18) Hemoglobin electrophoresis

- The physiological anemia of pregnancy can exacerbate the anemia from thalassemia

19) Menarche is imminent

- Thelarche $\rightarrow$ Pubarche $\rightarrow$ Menarche (follows pubic hair development)

20) Admission to the hospital for intravenous hydration and parenteral antiemetic therapy

- This patient likely has Hyperemesis Gravidarum (HG)
- HG tx: IV hydration + parenteral antiemetic therapy; Patient is unable to keep solids or liquids down $\rightarrow$ must be admitted for IV hydration
- The presence of ketones helps to differentiate HG from gestational N/V

21) Appendicitis

- Fever + leukocytosis + right-sided abdominal pain
- Hematuria and pyuria may occur if the inflamed appendix is close to the bladder or ureter

22) Cervical trauma

- Cervix is highly vascularized during pregnancy and can bleed more easily from trauma (eg, intercourse)

23) Fine-needle aspiration biopsy of the cyst

- Biopsy (often U/S guided) is required to confirm the diagnosis of a palpable mass
- FNA may be used for cystic/small masses; Core biopsy is used for solid masses

24) Hypoestrogenic state

- Hypoestrogenism → ↓ blood flow & ↓ collagen → ↓ epithelial elasticity and subsequent atrophy → thin, dry, easily denude urogenital epithelium → ↑ susceptibility to injury → vaginal/vulvar bleeding with minimal manipulation

25) Increased 5 α -reductase activity

- Excessive hair growth in the absence of lab abnormalities suggests ↑ 5 α -reductase activity (converts testosterone into DHT)
- Aromatase converts testosterone into estrogens (this patient has normal lab values)

26) Intravascular injection of anesthetic

- Inadvertent intravascular injection of Anesthetic → toxicity → tinnitus & metallic taste (from lidocaine), ↑ HR & BP (from epinephrine)

27) Anovulation

- Irregular periods suggest anovulation → ↑ unopposed estrogen → endometrial hyperplasia
- ↑ BMI → aromatization of androgens to estrogen

28) Follicle-stimulating hormone

- This patient likely has Turner syndrome (hypergonadotrophic hypogonadism)
- If a patient with primary amenorrhea doesn't have secondary sexual characteristics, the next best step is to measure FSH & LH levels
- GnRH is not a systemic hormone (inside hypophyseal portal blood system)

29) Suction and curettage

- This patient likely has a complete molar pregnancy (uterus size > gestational age, ↑↑↑ β -hCG); lack of fetal parts distinguishes from partial molar pregnancy
- Molar pregnancy is treated with D&C and the downtrend of β -hCG must be followed. Additionally, patient must be on a reliable contraceptive so that she does not become pregnant while here β -hCG downtrend is being followed
- Methotrexate is Tx for ectopic pregnancy if the patient is stable
- Misoprostol is a PGE analog used for abortion (not safe for use with molar pregnancy)

30) Decreased protein content in breast milk

- OCPs should not be given before 6 weeks postpartum
 - OCPs given before 3 weeks postpartum → ↑ risk of DVT
 - OCPs given before 6 weeks postpartum → ↓ protein content of breast milk (progesterone inhibits α -lactalbumin, the major protein found in breast milk)

31) Administration of Rh (D) immune globulin

- This patient is Rh $\ominus$ and unsensitized ($\ominus$ antibody test). This patient should be given Rhogam every time she comes into contact with fetal blood to keep her unsensitized

32) Punch biopsy of the affected areas

- This patient most likely has Lichen sclerosis
- Punch biopsy should be used to exclude malignancy
- Tx: potent corticosteroid ointment (this question asked for the most appropriate next step, not the best treatment option)

33) Congenital uterine anomalies

- This patient likely has some agenesis on the right. Development of a left sided paramesonephric (mullerian) duct → unicornuate uterus
- Palpable uterus on physical exam r/o Mullerian agenesis

34) *Neisseria gonorrhoeae*

- Gram $\ominus$ intracellular diplococci
- *Chlamydia* is difficult to stain/visualize

35) Umbilical cord compression

- This patients membranes have ruptures → loss of amniotic fluid → cord compression and variable decelerations
- Variable decelerations are due to cord compression, oligohydramnios, or cord prolapse

36) Testosterone

- Sertoli-Leydig (Arrhenoblastoma) - testosterone producing tumor of the ovary
- DHEAS is produced by adrenal glands

37) Pregnancy

- This patient hasn't had her period in 6 weeks, her uterus is enlarged, and she uses condoms inconsistently
- Thin clear vaginal discharge is physiologic in pregnancy

38) Severe preeclampsia

- This patient has new-onset HTN (systolic BP ≥ 140 or diastolic BP ≥ 90) with severe features (eg, thrombocytopenia, visual or cerebral sx's)

39) Atelectasis

- Surgery/Post-op care often → atelectasis + fever
- Lack of erythema at incision site r/o wound infection

40) Spontaneous disappearance of the lesions within 1 week

- Herpes simplex virus (HSV) generally resolves within a week of lesion development
- HSV recurrences become less frequent over time

41) Mastitis

- Mastitis - Unilateral, localized tenderness and erythema + fever
- Non-fluctuant nature R/O breast abscess

42) Decreased external urethral sphincter tone

- ↑ abdominal pressure (eg, cough, sneeze) → urine loss (**stress incontinence**)
- Multiparity or previous pelvic surgery → laxity of pelvic floor musculature → urethral sphincter insufficiency

43) Uteroplacental insufficiency

- The fetus is 33 weeks but fetal size is consistent with 30 week gestation = IUGR (uteroplacental insufficiency)
- Lupus → hyalinization of blood vessels → uteroplacental insufficiency
- U/S at 20 weeks showed a fetus with normal anatomy (the kidneys develop before this, thus renal agenesis would have been identified at that time)

44) Propylthiouracil therapy

- PTU is preferred treatment in thyroid storm because it inhibits T4 → T3 conversion
- For treating general hyperthyroidism, PTU is preferred for first trimester, Methimazole is preferred for second/third trimester (there is limited conflicting data on these drugs, and neither is absolutely contraindicated - it's a poor/old question)

45) Interstitial cystitis

- This patient has pain with urination (worse with bladder distention, improves upon urination) and bladder tenderness (anterior vaginal wall tenderness)
- U/A is within normal limits - rules out UTI
- No menorrhagia, dysmenorrhea - rules out adenomyosis
- No dysmenorrhea, dyschezia, or dyspareunia - rules out endometriosis

46) Type 2 diabetes mellitus

- ↑ sugar → overgrowth of yeast (particularly in the vagina)

47) Trimethoprim-sulfamethoxazole

- Prophylactic treatment is advocated for women with ≥2 UTIs in 6 months, or ≥3 UTIs in 12 months
- Use of TMP/SMX is reported to be effective and well tolerated, but does raise concerns about antimicrobial resistance

48) Normal postoperative course

- This patient's regional LAD and pain at incision site are normal post-op sxs
- Incisional seroma → collection/pocket of fluid (not found in this patient)

49) Abruptio placentae

- Classic presentation: 3rd trimester sudden-onset bleeding, abdominal/back pain, high-frequency low-intensity contractions, and hypertonic/tender uterus

50) Chorioamnionitis

- Intraamniotic infection (chorioamnionitis) is diagnosed with maternal fever, plus one or more of the following: fetal tachycardia (>160), maternal leukocytosis, or purulent amniotic fluid